

SOLVED PRACTICE WORKBOOK

Elderly and Senior Citizens - Solved Practice Workbook

UPSC complete learning session | Foundation to advanced | Concepts, evidence, practice and answer writing

Elderly and Senior Citizens	
REFRESHED TEACHING NAVIGATION — BASIC/CORE FIRST	
01	FOUNDATION — WHAT THE AGEING OWNER HOLDS AND HOW ITS DEMAND IS DIVIDED Technically, FOUNDATION — What the ageing owner holds and how
02	FOUNDATION — THE STATUTORY DEFINITIONS AND THE OBLIGATED PERSONS Section 2(b) of the Maintenance and Welfare of Parents and Senior
03	FOUNDATION — THE TRIBUNAL AND THE CEILING ON ITS ORDER Evidence chain: The tribunal and the ceiling on its order Qualified use:
04	CORE — THE PROPERTY REMEDY AND ITS CONDITION Evidence chain: The property-revocation remedy and its condition
05	CORE — THE OLD-AGE-HOME DIRECTION AND ITS PERFORMANCE Evidence chain: The old-age-home direction and the functionality
06	CORE — ENACTED LAW AGAINST PROPOSED AMENDMENT Evidence chain: Enacted law against proposed amendment Qualified
07	CORE — THE MINISTRY'S OWN ACCOUNT OF THE UMBRELLA SCHEME Evidence chain: The ministry's own account of the umbrella scheme
08	CORE — THE FIVE COMPONENTS RECORDED ON THAT PAGE Evidence chain: The five components recorded on that page Qualified
09	CORE — DEVICES, ELIGIBILITY AND THE HELPLINE AS A DETECTION POINT The device component is targeted assistance and not a universal
10	CORE — THE PENSION FLOOR AND THE GERIATRIC HEALTH ROUTE The owners record the national programme for the health care of the
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Original deterministic study visual; labels are explanatory, not textual quotations.

Source order: repository knowledge -> official current linkage -> clearly marked analysis. No fabricated PYQs or statistics.

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BASIC MCQS / REMEDIATION

Q1. Which statement correctly identifies What this owner holds and how its routed demand is divided?

A. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. B. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. C. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. D. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions.

Answer: A. Explanation: This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. The remaining options belong to different chronology, actor or analytical categories.

Q2. Which chronology card should be filed under What this owner holds and how its routed demand is divided?

A. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. B. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the

health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. C. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. D. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design.

Answer: B. Explanation: This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. The remaining options belong to different chronology, actor or analytical categories.

Q3. Which option preserves the source-bounded meaning of What this owner holds and how its routed demand is divided?

A. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. B. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. C. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. D. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as

achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed.

Answer: C. Explanation: This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. The remaining options belong to different chronology, actor or analytical categories.

Q4. Which statement avoids a close-option trap about What this owner holds and how its routed demand is divided?

A. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. B. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. C. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. D. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division.

Answer: D. Explanation: This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. The remaining options belong to different chronology, actor or analytical categories.

Q5. Which statement correctly identifies Who is a senior citizen and what maintenance means?

A. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions. B. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. C. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. D. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design.

Answer: A. Explanation: Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions. The remaining options belong to different chronology, actor or analytical categories.

Q6. Which chronology card should be filed under Who is a senior citizen and what maintenance means?

A. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. B. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions. C. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. D. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and

time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design.

Answer: B. Explanation: Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions. The remaining options belong to different chronology, actor or analytical categories.

Q7. Which option preserves the source-bounded meaning of Who is a senior citizen and what maintenance means?

A. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. B. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. C. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions. D. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law.

Answer: C. Explanation: Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions. The remaining options belong to different chronology, actor or analytical categories.

Q8. Which statement avoids a close-option trap about Who is a senior citizen and what maintenance means?

A. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. B. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were

introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. C. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. D. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions.

Answer: D. Explanation: Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions. The remaining options belong to different chronology, actor or analytical categories.

Q9. Which statement correctly identifies Who owes the obligation?

A. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. B. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. C. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. D. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute.

Answer: A. Explanation: The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and

not the parental link alone, is what carries the duty. The remaining options belong to different chronology, actor or analytical categories.

Q10. Which chronology card should be filed under Who owes the obligation?

A. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. B. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. C. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. D. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute.

Answer: B. Explanation: The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. The remaining options belong to different chronology, actor or analytical categories.

Q11. Which option preserves the source-bounded meaning of Who owes the obligation?

A. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. B. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. C. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the

common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. D. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed.

Answer: C. Explanation: The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. The remaining options belong to different chronology, actor or analytical categories.

Q12. Which statement avoids a close-option trap about Who owes the obligation?

A. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. B. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. C. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. D. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty.

Answer: D. Explanation: The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the

obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. The remaining options belong to different chronology, actor or analytical categories.

Q13. Which statement correctly identifies The tribunal and the ceiling on its order?

A. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. B. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. C. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. D. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law.

Answer: A. Explanation: The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. The remaining options belong to different chronology, actor or analytical categories.

Q14. Which chronology card should be filed under The tribunal and the ceiling on its order?

A. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. B. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. C. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens;

that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. D. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed.

Answer: B. Explanation: The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. The remaining options belong to different chronology, actor or analytical categories.

Q15. Which option preserves the source-bounded meaning of The tribunal and the ceiling on its order?

A. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. B. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. C. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. D. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced

penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law.

Answer: C. Explanation: The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. The remaining options belong to different chronology, actor or analytical categories.

Q16. Which statement avoids a close-option trap about The tribunal and the ceiling on its order?

A. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. B. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. C. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. D. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design.

Answer: D. Explanation: The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy

accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. The remaining options belong to different chronology, actor or analytical categories.

Q17. Which statement correctly identifies The property-revocation remedy and its condition?

A. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. B. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. C. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. D. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed.

Answer: A. Explanation: The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. The remaining options belong to different chronology, actor or analytical categories.

Q18. Which chronology card should be filed under The property-revocation remedy and its condition?

A. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. B. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation

right has overstated the statute. C. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. D. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed.

Answer: B. Explanation: The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. The remaining options belong to different chronology, actor or analytical categories.

Q19. Which option preserves the source-bounded meaning of The property-revocation remedy and its condition?

A. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. B. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. C. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and

physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. D. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim.

Answer: C. Explanation: The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. The remaining options belong to different chronology, actor or analytical categories.

Q20. Which statement avoids a close-option trap about The property-revocation remedy and its condition?

A. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. B. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. C. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. D. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute.

Answer: D. Explanation: The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that

presents it as a general revocation right has overstated the statute. The remaining options belong to different chronology, actor or analytical categories.

Q21. Which statement correctly identifies The old-age-home direction and the functionality question?

A. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. B. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. C. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. D. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator.

Answer: A. Explanation: The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. The remaining options belong to different chronology, actor or analytical categories.

Q22. Which chronology card should be filed under The old-age-home direction and the functionality question?

A. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. B. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. C. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. D. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator.

Answer: B. Explanation: The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. The remaining options belong to different chronology, actor or analytical categories.

Q23. Which option preserves the source-bounded meaning of The old-age-home direction and the functionality question?

A. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. B. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter,

food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. C. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. D. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim.

Answer: C. Explanation: The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. The remaining options belong to different chronology, actor or analytical categories.

Q24. Which statement avoids a close-option trap about The old-age-home direction and the functionality question?

A. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. B. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. C. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard. D. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed.

Answer: D. Explanation: The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. The remaining options belong to different chronology, actor or analytical categories.

Q25. Which statement correctly identifies Enacted law against proposed amendment?

A. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. B. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. C. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. D. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator.

Answer: A. Explanation: The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. The remaining options belong to different chronology, actor or analytical categories.

Q26. Which chronology card should be filed under Enacted law against proposed amendment?

A. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. B. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. C. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. D. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists.

Answer: B. Explanation: The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. The remaining options belong to different chronology, actor or analytical categories.

Q27. Which option preserves the source-bounded meaning of Enacted law against proposed amendment?

A. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. B. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance

route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. C. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. D. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard.

Answer: C. Explanation: The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. The remaining options belong to different chronology, actor or analytical categories.

Q28. Which statement avoids a close-option trap about Enacted law against proposed amendment?

A. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard. B. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. C. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. D. The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law.

Answer: D. Explanation: The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying

relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. The remaining options belong to different chronology, actor or analytical categories.

Q29. Which statement correctly identifies The ministry's own account of the umbrella scheme?

A. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. B. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. C. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. D. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator.

Answer: A. Explanation: The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. The remaining options belong to different chronology, actor or analytical categories.

Q30. Which chronology card should be filed under The ministry's own account of the umbrella scheme?

A. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. B. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. C. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard. D. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists.

Answer: B. Explanation: The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. The remaining options belong to different chronology, actor or analytical categories.

Q31. Which option preserves the source-bounded meaning of The ministry's own account of the umbrella scheme?

A. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. B. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency

response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. C. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. D. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard.

Answer: C. Explanation: The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. The remaining options belong to different chronology, actor or analytical categories.

Q32. Which statement avoids a close-option trap about The ministry's own account of the umbrella scheme?

A. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. B. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself. C. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard. D. The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice

and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed.

Answer: D. Explanation: The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. The remaining options belong to different chronology, actor or analytical categories.

Q33. Which statement correctly identifies The five components recorded on that page?

A. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. B. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. C. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. D. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard.

Answer: A. Explanation: The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. The remaining options belong to different chronology, actor or analytical categories.

Q34. Which chronology card should be filed under The five components recorded on that page?

A. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard. B. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. C. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. D. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes.

Answer: B. Explanation: The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever

applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. The remaining options belong to different chronology, actor or analytical categories.

Q35. Which option preserves the source-bounded meaning of The five components recorded on that page?

A. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard. B. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. C. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. D. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself.

Answer: C. Explanation: The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. The remaining options belong to different chronology, actor or analytical categories.

Q36. Which statement avoids a close-option trap about The five components recorded on that page?

A. The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. B. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself. C. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. D. The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator.

Answer: D. Explanation: The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. The remaining options belong to different chronology, actor or analytical categories.

Q37. Which statement correctly identifies What the assistive-device component actually requires?

A. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. B. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. C. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. D. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard.

Answer: A. Explanation: The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. The remaining options belong to different chronology, actor or analytical categories.

Q38. Which chronology card should be filed under What the assistive-device component actually requires?

A. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself. B. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. C. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term

care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. D. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard.

Answer: B. Explanation: The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. The remaining options belong to different chronology, actor or analytical categories.

Q39. Which option preserves the source-bounded meaning of What the assistive-device component actually requires?

A. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. B. The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. C. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. D. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself.

Answer: C. Explanation: The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. The remaining options belong to different chronology, actor or analytical categories.

Q40. Which statement avoids a close-option trap about What the assistive-device component actually requires?

A. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself. B. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. C. The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. D. The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim.

Answer: D. Explanation: The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. The remaining options belong to different chronology, actor or analytical categories.

Q41. Which statement correctly identifies The national helpline and what a helpline can do?

A. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. B. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself. C. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard. D. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they

record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes.

Answer: A. Explanation: The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. The remaining options belong to different chronology, actor or analytical categories.

Q42. Which chronology card should be filed under The national helpline and what a helpline can do?

A. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. B. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. C. The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. D. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself.

Answer: B. Explanation: The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. The remaining options belong to different chronology, actor or analytical categories.

Q43. Which option preserves the source-bounded meaning of The national helpline and what a helpline can do?

A. The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. B. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself. C. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. D. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse.

Answer: C. Explanation: The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. The remaining options belong to different chronology, actor or analytical categories.

Q44. Which statement avoids a close-option trap about The national helpline and what a helpline can do?

A. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. B. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. C. The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that

is mostly financial and relational. D. The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists.

Answer: D. Explanation: The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. The remaining options belong to different chronology, actor or analytical categories.

Q45. Which statement correctly identifies The old-age pension floor and how to use it?

A. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard. B. The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. C. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself. D. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes.

Answer: A. Explanation: The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard. The remaining options belong to different chronology, actor or analytical categories.

Q46. Which chronology card should be filed under The old-age pension floor and how to use it?

A. The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. B. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard. C. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself. D. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse.

Answer: B. Explanation: The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard. The remaining options belong to different chronology, actor or analytical categories.

Q47. Which option preserves the source-bounded meaning of The old-age pension floor and how to use it?

A. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. B. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. C. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard. D. The owners set out four kinds of elder abuse rather than one: physical abuse

including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational.

Answer: C. Explanation: The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard. The remaining options belong to different chronology, actor or analytical categories.

Q48. Which statement avoids a close-option trap about The old-age pension floor and how to use it?

A. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. B. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. C. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts. D. The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard.

Answer: D. Explanation: The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard. The remaining options belong to different chronology, actor or analytical categories.

Q49. Which statement correctly identifies The geriatric limb of the health system?

A. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. B. The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. C. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. D. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself.

Answer: A. Explanation: The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. The remaining options belong to different chronology, actor or analytical categories.

Q50. Which chronology card should be filed under The geriatric limb of the health system?

A. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. B. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. C. The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different

remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. D. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse.

Answer: B. Explanation: The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. The remaining options belong to different chronology, actor or analytical categories.

Q51. Which option preserves the source-bounded meaning of The geriatric limb of the health system?

A. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. B. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. C. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. D. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts.

Answer: C. Explanation: The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. The remaining options belong to different chronology, actor or analytical categories.

Q52. Which statement avoids a close-option trap about The geriatric limb of the health system?

A. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts.

B. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population.

C. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for.

D. The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes.

Answer: D. Explanation: The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. The remaining options belong to different chronology, actor or analytical categories.

Q53. Which statement correctly identifies The evidence source for ageing and its discipline?

A. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself.

B. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse.

C. The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that

is mostly financial and relational. D. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for.

Answer: A. Explanation: The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself. The remaining options belong to different chronology, actor or analytical categories.

Q54. Which chronology card should be filed under The evidence source for ageing and its discipline?

A. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. B. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself. C. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. D. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts.

Answer: B. Explanation: The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself. The remaining options belong to different chronology, actor or analytical categories.

Q55. Which option preserves the source-bounded meaning of The evidence source for ageing and its discipline?

A. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. B. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population. C. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself. D. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts.

Answer: C. Explanation: The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself. The remaining options belong to different chronology, actor or analytical categories.

Q56. Which statement avoids a close-option trap about The evidence source for ageing and its discipline?

A. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population. B. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. C. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of

disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts. D. The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself.

Answer: D. Explanation: The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself. The remaining options belong to different chronology, actor or analytical categories.

Q57. Which statement correctly identifies Elder abuse as a typology, not a single act?

A. The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. B. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts. C. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. D. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for.

Answer: A. Explanation: The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. The remaining options belong to different chronology, actor or analytical categories.

Q58. Which chronology card should be filed under Elder abuse as a typology, not a single act?

A. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. B. The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. C. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts. D. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population.

Answer: B. Explanation: The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. The remaining options belong to different chronology, actor or analytical categories.

Q59. Which option preserves the source-bounded meaning of Elder abuse as a typology, not a single act?

A. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. B. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population. C. The owners set out four kinds of elder abuse rather than one: physical abuse

including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. D. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts.

Answer: C. Explanation: The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. The remaining options belong to different chronology, actor or analytical categories.

Q60. Which statement avoids a close-option trap about Elder abuse as a typology, not a single act?

A. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population. B. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. C. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. D. The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational.

Answer: D. Explanation: The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. The remaining options belong to different chronology, actor or analytical categories.

Q61. Which statement correctly identifies Why elder abuse is under-reported?

A. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. B. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. C. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts. D. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population.

Answer: A. Explanation: The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. The remaining options belong to different chronology, actor or analytical categories.

Q62. Which chronology card should be filed under Why elder abuse is under-reported?

A. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population. B. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for

policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. C. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. D. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts.

Answer: B. Explanation: The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. The remaining options belong to different chronology, actor or analytical categories.

Q63. Which option preserves the source-bounded meaning of Why elder abuse is under-reported?

A. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. B. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. C. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. D. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in

health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population.

Answer: C. Explanation: The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. The remaining options belong to different chronology, actor or analytical categories.

Q64. Which statement avoids a close-option trap about Why elder abuse is under-reported?

A. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. B. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. C. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions. D. The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse.

Answer: D. Explanation: The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. The remaining options belong to different chronology, actor or analytical categories.

Q65. Which statement correctly identifies The care economy under demographic and household change?

A. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. B. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. C. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population. D. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts.

Answer: A. Explanation: The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. The remaining options belong to different chronology, actor or analytical categories.

Q66. Which chronology card should be filed under The care economy under demographic and household change?

A. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. B. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women,

typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. C. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population. D. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division.

Answer: B. Explanation: The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. The remaining options belong to different chronology, actor or analytical categories.

Q67. Which option preserves the source-bounded meaning of The care economy under demographic and household change?

A. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. B. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. C. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. D. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and

medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions.

Answer: C. Explanation: The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. The remaining options belong to different chronology, actor or analytical categories.

Q68. Which statement avoids a close-option trap about The care economy under demographic and household change?

A. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. B. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions. C. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. D. The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for.

Answer: D. Explanation: The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. The remaining options belong to different chronology, actor or analytical categories.

Q69. Which statement correctly identifies Active ageing as the policy frame?

A. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts.

B. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner.

C. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population.

D. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division.

Answer: A. Explanation: The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts. The remaining options belong to different chronology, actor or analytical categories.

Q70. Which chronology card should be filed under Active ageing as the policy frame?

A. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions.

B. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts.

C. The audited 2018-2023 Mains

routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. D. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division.

Answer: B. Explanation: The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts. The remaining options belong to different chronology, actor or analytical categories.

Q71. Which option preserves the source-bounded meaning of Active ageing as the policy frame?

A. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. B. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. C. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts. D. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions.

Answer: C. Explanation: The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts. The remaining options belong to different chronology, actor or analytical categories.

Q72. Which statement avoids a close-option trap about Active ageing as the policy frame?

A. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. B. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. C. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions. D. The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts.

Answer: D. Explanation: The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts. The remaining options belong to different chronology, actor or analytical categories.

Q73. Which statement correctly identifies The official demographic series and the projection boundary?

A. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population. B. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in

old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. C. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. D. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions.

Answer: A. Explanation: The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population. The remaining options belong to different chronology, actor or analytical categories.

Q74. Which chronology card should be filed under The official demographic series and the projection boundary?

A. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions. B. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population. C. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. D. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior

citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty.

Answer: B. Explanation: The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population. The remaining options belong to different chronology, actor or analytical categories.

Q75. Which option preserves the source-bounded meaning of The official demographic series and the projection boundary?

A. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. B. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. C. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population. D. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions.

Answer: C. Explanation: The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population. The remaining options belong to different chronology, actor or analytical categories.

Q76. Which statement avoids a close-option trap about The official demographic series and the projection boundary?

A. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. B. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. C. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. D. The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population.

Answer: D. Explanation: The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population. The remaining options belong to different chronology, actor or analytical categories.

Q77. Which statement correctly identifies The verified previous-year ownership stated precisely?

A. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. B. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions. C. This owner holds the ageing half of the social-protection argument: the statutory maintenance obligation and its tribunal, the property-revocation remedy, the district old-age-home direction, the umbrella welfare scheme of the social-justice ministry with its named components, the assistance floor in old-age pension, the geriatric limb of the health system, the elder-abuse typology and the care economy; its ownership

record is verified rather than empty, because the audited 2018-2023 Mains routing ledger routes the 2020 General Studies Paper II question on health policies for geriatric and maternal care to this owner and to the health-systems owner jointly and records the ownership as cross-cutting, so this package solves the geriatric limb of that demand and states expressly that the maternal limb belongs to the health-systems owner, which is exactly how the Basic owner's answer architecture records the division. D. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty.

Answer: A. Explanation: The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. The remaining options belong to different chronology, actor or analytical categories.

Q78. Which chronology card should be filed under The verified previous-year ownership stated precisely?

A. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. B. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. C. Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions. D. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design.

Answer: B. Explanation: The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership

recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. The remaining options belong to different chronology, actor or analytical categories.

Q79. Which option preserves the source-bounded meaning of The verified previous-year ownership stated precisely?

A. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. B. The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. C. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. D. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design.

Answer: C. Explanation: The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. The remaining options belong to different chronology, actor or analytical categories.

Q80. Which statement avoids a close-option trap about The verified previous-year ownership stated precisely?

A. The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. B. The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. C. The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. D. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner.

Answer: D. Explanation: The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. The remaining options belong to different chronology, actor or analytical categories.

PYQS AND ANSWER PRACTICE

VERIFIED PYQ OWNERSHIP AUDIT

This owner's previous-year record is a verified one rather than a zero, and it is stated precisely together with the boundary of what is claimed. The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand here, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, with the directive Discuss, ten marks and one hundred and fifty words, and it records the ownership as cross-cutting between this owner and the health-systems and universal-health-coverage owner because the question names two distinct care domains. The Basic owner's answer architecture records the division in the same terms, stating that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership. This package therefore solves the geriatric limb of that demand with a model answer written to the repository's examiner-grade standard using the claim, named evidence, analysis and qualification pattern, and states expressly in

the demand card that the maternal-care limb belongs to the health-systems owner and is not claimed here, so that the same question is not double-counted as two independent ownerships. No other Mains or objective demand is claimed: the audited 2024-2025 Mains and Prelims routing ledgers route none to this owner, both the Basic and the Advanced owner state independently that no confirmed direct 2024 or 2025 General Studies Paper II Mains question specifically on elderly welfare exists in the audited local texts, and the 2018 General Studies Paper IV case study involving a district officer and an elderly couple seeking a senior-citizen healthcare scheme is routed by the audited ledger to the Ethics case-study and public-service-values owners, so it is not claimed here either. Alongside the solved demand card, the package adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation. Each model solution opens with its analytical claim so that the evidence which follows does argumentative rather than decorative work. The locally held OCR-searchable official General Studies papers were read only to confirm the printed wording of the routed Mains demand; no question was invented from them, no marking scheme or page precision was imported and no official answer key is claimed for any question anywhere in this package.

Demand decoding: The directive **answer** requires a direct position on “VERIFIED PYQ OWNERSHIP AUDIT”, every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: The answer must resolve the Social Justice demand in “VERIFIED PYQ OWNERSHIP AUDIT”.

Analytical body:

1. **Claim and named evidence:** Define the vulnerable group, deprivation, right or policy concept and distinguish the nearest legal and measured category. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction.
Qualification: State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
2. **Claim and named evidence:** Identify the constitutional, statutory, judicial or executive basis and the competent Union, state or local institution. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction.
Qualification: State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
3. **Claim and named evidence:** Map rights-holder, household/community context, duty-bearer, frontline worker, provider and accountability forum. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction.
Qualification: State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
4. **Claim and named evidence:** Trace identification, eligibility, documentation, finance, access, portability, grievance, appeal and correction. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction.
Qualification: State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
5. **Claim and named evidence:** Use a named Indian law, judgment, scheme, institution or official dataset with source, date, denominator and status. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction.
Qualification: State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
6. **Claim and named evidence:** Test intersectionality, regional variation, stigma, accessibility, privacy, fiscal adequacy, exclusion and causal limits. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction.
Qualification: State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: The answer must resolve the Social Justice demand in “VERIFIED PYQ OWNERSHIP AUDIT”.

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

Why this earns marks: The answer obeys the directive, explains mechanisms rather than listing schemes, uses named India-centric evidence and preserves legal, institutional, group, eligibility, implementation, fiscal, data and outcome distinctions.

How to improve this answer: For “VERIFIED PYQ OWNERSHIP AUDIT”, replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

OWNER PYQ LEDGER EXTRACTS

9. PYQ application

- ANALYSIS: There is no confirmed direct 2024 or 2025 GS-II Mains PYQ specifically on elderly welfare in the audited local PYQ texts. However, questions on social security, welfare and demographic transition may intersect with this topic. A strong answer should cite the 2007 Act's maintenance-obligation and property-revocation architecture, AVYAY's umbrella structure, and the under-reporting challenge in elder abuse.

Historical PYQ Integration (2018-2023)

Status: Question-level PYQ demand is integrated into this owner. **Provenance:** Audited local official-paper routing ledgers: _PYQ-ROUTING-MAINS-GS1-GS2-ESSAY-2018-2023.md.

- Years represented: 2020
- Paper(s): GS-II
- Routed question demands: 1

Year	Paper	Q	PYQ demand (neutral rendering)	Directive / format	Source status	Owner requirement
2020	GS-II	6	Health policies for geriatric and maternal care	Discuss · 10 marks · 150 words	Cross-cutting Core ownership	Prepare context, core dimensions, evidence/examples, counterpoint and a concise conclusion.

What this owner must now support

- Health policies for geriatric and maternal care

The table integrates the examinable demand and paper metadata. It does not turn an unkeyed objective question into a solved answer, and it does not claim that lexical presence alone proves full conceptual sufficiency.

10. PYQ-based analytical application

- ANALYSIS: There is no confirmed direct 2024 or 2025 GS-II Mains PYQ specifically on elderly welfare in the audited local PYQ texts. A strong Mains answer should: (a) cite the 2007 Act's architecture (maintenance, Tribunals, property remedy) and its limitations, (b) distinguish enacted law from pending 2025 Private Members' Bills, (c) map AVYAY's components, (d) flag the elder-abuse under-reporting challenge, and (e) cross-link geriatric-health integration to the health-system topic.

Historical PYQ Integration (2018-2023)

Status: Question-level PYQ demand is integrated into this owner. **Provenance:** Audited local official-paper routing ledgers: _PYQ-ROUTING-MAINS-GS1-GS2-ESSAY-2018-2023.md.

- **Years represented:** 2020
- **Paper(s):** GS-II
- **Routed question demands:** 1

Year	Paper	Q	PYQ demand (neutral rendering)	Directive / format	Source status	Owner requirement
2020	GS-II	6	Health policies for geriatric and maternal care	Discuss · 10 marks · 150 words	Cross-cutting; the question names two distinct care domains	Prepare context, core dimensions, evidence/examples, counterpoint and a concise conclusion.

What this owner must now support

- Health policies for geriatric and maternal care

The table integrates the examinable demand and paper metadata. It does not turn an unkeyed objective question into a solved answer, and it does not claim that lexical presence alone proves full conceptual sufficiency.

PYQ DEMAND CARD 1 - 2020 General Studies Paper II

Demand: Health policies for geriatric and maternal care, with the directive Discuss, ten marks and one hundred and fifty words, exactly as recorded in the audited 2018-2023 Mains routing ledger, where ownership is recorded as cross-cutting between this owner and the health-systems and universal-health-coverage owner because the question names two distinct care domains.

Status: Routed to this owner jointly with the health-systems owner by the audited 2018-2023 Mains routing ledger, and recorded in the Basic owner's answer architecture as an ownership in which the elderly-care half belongs here and supersedes the older Advanced ownership. The model solution below therefore answers the geriatric limb in full and states the maternal limb as belonging to the health-systems owner rather than reproducing it; no official marking scheme, model answer or page precision is claimed, and the answer is this repository's own examiner-grade solution.

Model solution: Claim: geriatric care is the point at which India's health architecture is tested not by its ability to treat an episode but by its ability to sustain continuity, so a geriatric health policy that is built around hospitalisation will look adequate on paper and fail in the household. Named evidence and example: the national programme for the health care of the elderly, which is the designated geriatric-care route inside the public-health system; the age-based hospitalisation route under the national health-assurance scheme, which covers every citizen aged seventy and above and therefore removes the means test at the oldest ages while addressing inpatient episodes only; the recorded limitation that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs, which are chronic rather than episodic, are managed informally or in tertiary hospitals; the umbrella welfare scheme of the social-justice ministry, whose own official page lists health care and nutrition among its developmental objectives and whose components include senior-citizen homes, a State action plan, assisted-living devices conforming to Bureau of Indian Standards specifications for persons aged sixty and above with age-related infirmity who are below the poverty line or have family income up to fifteen thousand rupees a month, a productive-engagement portal and an aged-care products engine; the national helpline for senior citizens as a grievance and emergency channel; the assistance floor of two hundred rupees a month for eligible below-poverty-line persons aged sixty to seventy-nine and five hundred rupees for those aged eighty and above, which shapes what an elderly household can spend out of pocket; and the demographic series recorded on that same official page, showing elderly persons rising from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, with projections of 14.3 crore in 2021 and 17.3 crore in 2026. Analysis: these instruments sit in three different administrative silos, and the joins between them are where the elderly person is lost, because a hospitalisation entitlement does not purchase outpatient follow-up, rehabilitation after a fracture, palliative care in a terminal illness or long-term care in cognitive decline; an assistive device restores function but is delivered on a means test and a camp schedule; an income floor of a few hundred rupees cannot finance a continuing drug

regimen; and the informal carer who fills all of these gaps is typically a woman whose unpaid labour is invisible to every one of the schemes. The policy question is therefore integration rather than creation, and it requires geriatric competence at the primary level, a defined continuum from prevention to palliative care, and coordination between the social-justice ministry that owns the welfare components and the health ministry that owns the clinical ones. Qualification: the maternal-care limb of this question is owned by the health-systems and universal-health-coverage owner and is answered there through the reproductive and child-health route, so it is deliberately not reproduced here; and no coverage, enrolment, claim, facility or expenditure figure is asserted, while the 2021 and 2026 elderly-population figures are recorded projections on the scheme page and not enumerated counts. Why this earns marks: it answers the directive Discuss by taking a defensible position on continuity against episode, uses exact programme names, eligibility rules, statutory ages, assistance amounts and a dated demographic series as argumentative evidence, locates the failure at named joins rather than asserting general weakness, and closes with a qualified conclusion that respects both the cross-cutting ownership of the question and the limits of the available data.

Demand decoding: The directive **answer** requires a direct position on “PYQ DEMAND CARD 1 - 2020 General Studies Paper II”, every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: Claim: geriatric care is the point at which India's health architecture is tested not by its ability to treat an episode but by its ability to sustain continuity, so a geriatric health policy that is built around hospitalisation will look adequate on paper and fail in the household. Named evidence and example: the national programme for the health care of the elderly, which is the designated geriatric-care route inside the public-health system; the age-based hospitalisation route under the national health-assurance scheme, which covers every citizen aged seventy and above and therefore removes the means test at the oldest ages while addressing inpatient episodes only; the recorded limitation that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs, which are chronic rather than episodic, are managed informally or in tertiary hospitals; the umbrella welfare scheme of the social-justice ministry, whose own official page lists health care and nutrition among its developmental objectives and whose components include senior-citizen homes, a State action plan, assisted-living devices conforming to Bureau of Indian Standards specifications for persons aged sixty and above with age-related infirmity who are below the poverty line or have family income up to fifteen thousand rupees a month, a productive-engagement portal and an aged-care products engine; the national helpline for senior citizens as a grievance and emergency channel; the assistance floor of two hundred rupees a month for eligible below-poverty-line persons aged sixty to seventy-nine and five hundred rupees for those aged eighty and above, which shapes what an elderly household can spend out of pocket; and the demographic series recorded on that same official page, showing elderly persons rising from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, with projections of 14.3 crore in 2021 and 17.3 crore in 2026. Analysis: these instruments sit in three different administrative silos, and the joins between them are where the elderly person is lost, because a hospitalisation entitlement does not purchase outpatient follow-up, rehabilitation after a fracture, palliative care in a terminal illness or long-term care in cognitive decline; an assistive device restores function but is delivered on a means test and a camp schedule; an income floor of a few hundred rupees cannot finance a continuing drug regimen; and the informal carer who fills all of these gaps is typically a woman whose unpaid labour is invisible to every one of the schemes. The policy question is therefore integration rather than creation, and it requires geriatric competence at the primary level, a defined continuum from prevention to palliative care, and coordination between the social-justice ministry that owns the welfare components and the health ministry that owns the clinical ones. Qualification: the maternal-care limb of this question is owned by the health-systems and universal-health-coverage owner and is answered there through the reproductive and child-health route, so it is deliberately not reproduced here; and no coverage, enrolment, claim, facility or expenditure figure is asserted, while the 2021 and 2026 elderly-population figures are recorded projections on the scheme page and not enumerated counts. Why this earns marks: it answers the directive Discuss by taking a defensible position on continuity against episode, uses exact programme names, eligibility rules, statutory ages, assistance amounts and a dated demographic series as argumentative evidence, locates the failure at named joins rather than asserting general weakness, and closes with a qualified conclusion that respects both the cross-cutting ownership of the question and the limits of the available data.

Analytical body:

1. **Claim and named evidence:** PYQ DEMAND CARD 1 - 2020 General Studies Paper II **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: Claim: geriatric care is the point at which India's health architecture is tested not by its ability to treat an episode but by its ability to sustain continuity, so a geriatric health policy that is built around hospitalisation will look adequate on paper and fail in the household. Named evidence and example: the national programme for the health care of the elderly, which is the designated geriatric-care route inside the public-health system; the age-based hospitalisation route under the national health-assurance scheme, which covers every citizen aged seventy and above and therefore removes the means test at the oldest ages while addressing inpatient episodes only; the recorded limitation that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs, which are chronic rather than episodic, are managed informally or in tertiary hospitals; the umbrella welfare scheme of the social-justice ministry, whose own official page lists health care and nutrition among its developmental objectives and whose components include senior-citizen homes, a State action plan, assisted-living devices conforming to Bureau of Indian Standards specifications for persons aged sixty and above with age-related infirmity who are below the poverty line or have family income up to fifteen thousand rupees a month, a productive-engagement portal and an aged-care products engine; the national helpline for senior citizens as a grievance and emergency channel; the assistance floor of two hundred rupees a month for eligible below-poverty-line persons aged sixty to seventy-nine and five hundred rupees for those aged eighty and above, which shapes what an elderly household can spend out of pocket; and the demographic series recorded on that same official page, showing elderly persons rising from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, with projections of 14.3 crore in 2021 and 17.3 crore in 2026. Analysis: these instruments sit in three different administrative silos, and the joins between them are where the elderly person is lost, because a hospitalisation entitlement does not purchase outpatient follow-up, rehabilitation after a fracture, palliative care in a terminal illness or long-term care in cognitive decline; an assistive device restores function but is delivered on a means test and a camp schedule; an income floor of a few hundred rupees cannot finance a continuing drug regimen; and the informal carer who fills all of these gaps is typically a woman whose unpaid labour is invisible to every one of the schemes. The policy question is therefore integration rather than creation, and it requires geriatric competence at the primary level, a defined continuum from prevention to palliative care, and coordination between the social-justice ministry that owns the welfare components and the health ministry that owns the clinical ones. Qualification: the maternal-care limb of this question is owned by the health-systems and universal-health-coverage owner and is answered there through the reproductive and child-health route, so it is deliberately not reproduced here; and no coverage, enrolment, claim, facility or expenditure figure is asserted, while the 2021 and 2026 elderly-population figures are recorded projections on the scheme page and not enumerated counts. Why this earns marks: it answers the directive Discuss by taking a defensible position on continuity against episode, uses exact programme names, eligibility rules, statutory ages, assistance amounts and a dated demographic series as argumentative evidence, locates the failure at named joins rather than asserting general weakness, and closes with a qualified conclusion that respects both the cross-cutting ownership of the question and the limits of the available data.

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

How to improve this answer: For "PYQ DEMAND CARD 1 - 2020 General Studies Paper II", replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

ORIGINAL MAINS 1 - 10 MARKS

Question: Examine the key provisions of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 for protecting the rights of senior citizens. Answer in about 150 words.

Model thesis: The Act creates a remedy rather than a benefit, so the examination must state the definition, the obligated persons, the tribunal with its ceiling and time limit, the conditional property remedy and the old-age-home direction, and then say where each stops.

Claim -> named evidence -> analysis -> qualification:

- Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions.
- The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty.
- The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design.
- The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute.

Qualified conclusion: The Act creates a remedy rather than a benefit, so the examination must state the definition, the obligated persons, the tribunal with its ceiling and time limit, the conditional property remedy and the old-age-home direction, and then say where each stops.

Demand decoding: The directive **examine** requires a direct position on "Examine the key provisions of the Maintenance and Welfare of Parents and Senior Citizens Act,...", every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: The Act creates a remedy rather than a benefit, so the examination must state the definition, the obligated persons, the tribunal with its ceiling and time limit, the conditional property remedy and the old-age-home direction, and then say where each stops.

Analytical body:

1. **Claim and named evidence:** Section 2(h) of the Maintenance and Welfare of Parents and Senior Citizens Act, 2007 defines a senior citizen as a citizen of India who has attained the age of sixty years, and the Act defines maintenance as the provision of food, clothing, residence and medical attendance and treatment; the owners record that these are statutory definitions and not descriptive conveniences, so an answer must use the age of sixty as the legal threshold and must state maintenance as the four-part content of the obligation rather than as a general duty of care, because both the tribunal's jurisdiction and the content of its order follow from these definitions. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline

implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

2. **Claim and named evidence:** The owners record that under the 2007 Act the obligation to maintain a parent or a senior citizen falls on children and on relatives, relatives being those who are or who will be the legal heirs of the senior citizen's property, and they mark the common error explicitly: an answer that confines the obligation to children has narrowed a statute that deliberately reaches beyond the parent-child relationship in order to cover childless senior citizens whose property will pass to another relative, so the heirship link, and not the parental link alone, is what carries the duty. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
3. **Claim and named evidence:** The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
4. **Claim and named evidence:** The owners record the remedy under Section 23 with its condition intact: where a senior citizen has transferred property by gift or otherwise subject to the condition that the transferee provide basic amenities and physical needs, and the transferee refuses or fails to do so, the transfer may at the option of the senior citizen be declared void; the discipline is that the condition is part of the remedy and not a formality, so this is not an automatic power to undo any later family dispute over property, and an answer that presents it as a general revocation right has overstated the statute. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: The Act creates a remedy rather than a benefit, so the examination must state the definition, the obligated persons, the tribunal with its ceiling and time limit, the conditional property remedy and the old-age-home direction, and then say where each stops.

Executable exam-length answer / compression plan: For 10 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

Why this earns marks: The answer obeys the directive, explains mechanisms rather than listing schemes, uses named India-centric evidence and preserves legal, institutional, group, eligibility, implementation, fiscal, data and outcome distinctions.

How to improve this answer: For "Examine the key provisions of the Maintenance and Welfare of Parents and Senior Citizens Act,...", replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

ORIGINAL MAINS 2 - 10 MARKS

Question: Distinguish enacted law from proposed reform in India's elderly-welfare framework, and explain why the distinction matters for an answer. Answer in about 150 words.

Model thesis: Status errors here are decisive, so the distinction must set the 2007 Act against a lapsed 2019 Bill and unenacted 2025 Private Members' Bills, and must show what a candidate would wrongly assert if the distinction were dropped.

Claim -> named evidence -> analysis -> qualification:

- The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law.
- The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design.
- The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed.
- The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner.

Qualified conclusion: Status errors here are decisive, so the distinction must set the 2007 Act against a lapsed 2019 Bill and unenacted 2025 Private Members' Bills, and must show what a candidate would wrongly assert if the distinction were dropped.

Demand decoding: The directive **explain** requires a direct position on "Distinguish enacted law from proposed reform in India's elderly-welfare framework, and...", every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: Status errors here are decisive, so the distinction must set the 2007 Act against a lapsed 2019 Bill and unenacted 2025 Private Members' Bills, and must show what a candidate would wrongly assert if the distinction were dropped.

Analytical body:

1. **Claim and named evidence:** The owners keep the legislative record exact: an amendment Bill was first introduced in the Lok Sabha in December 2019 proposing an expanded definition of a life of dignity, enhanced penalties for abandonment and mechanisms such as direct deduction of the maintenance amount from a paying relative's income, and that Bill lapsed with the dissolution of that Lok Sabha and was never enacted; Private Members' Bills were introduced in the Lok Sabha and the Rajya Sabha in 2025 and, as at 21 July 2026, neither has been enacted, so the law in force is the 2007 Act and any broader dignity definition, enhanced penalty or income-deduction mechanism must be labelled as proposed and never as current law. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
2. **Claim and named evidence:** The Act establishes a Maintenance Tribunal in every sub-division as a summary adjudicatory body to receive applications from senior citizens or authorised persons and ordinarily to decide within ninety days, extendable in exceptional circumstances, and the owners record that the allowance prescribed by the State cannot exceed the Act's ceiling of ten thousand rupees per month; the two facts must travel together in an answer, because the summary and time-bound character is what makes the remedy accessible while the monetary ceiling is what limits its adequacy in a high-cost setting, and a candidate who cites one without the other has described only half of the design. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
3. **Claim and named evidence:** The Act directs State Governments to establish and maintain at least one old-age home in each district with capacity for at least one hundred and fifty indigent senior citizens, and the owners immediately separate the direction from its performance: the statutory instruction fixes the number and the capacity, while whether such homes exist and function is the implementation question, so an answer may state the direction as law and must not state coverage as achieved, and the honest formulation is that the norm is statutory and its fulfilment is a State-level fact to be verified rather than assumed. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
4. **Claim and named evidence:** The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: Status errors here are decisive, so the distinction must set the 2007 Act against a lapsed 2019 Bill and unenacted 2025 Private Members' Bills, and must show what a candidate would wrongly assert if the distinction were dropped.

Executable exam-length answer / compression plan: For 10 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility,

portability, grievance, denominator, fiscal and causal limits.

Why this earns marks: The answer obeys the directive, explains mechanisms rather than listing schemes, uses named India-centric evidence and preserves legal, institutional, group, eligibility, implementation, fiscal, data and outcome distinctions.

How to improve this answer: For “Distinguish enacted law from proposed reform in India’s elderly-welfare framework, and...”, replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

ORIGINAL MAINS 3 - 15 MARKS

Question: Critically examine the claim that India’s elderly-welfare architecture is fragmented across law, welfare schemes and health services. Answer in about 250 words.

Model thesis: Fragmentation is an integration failure and not an absence of instruments, so the examination must map the statutory remedy, the five scheme components and the geriatric health route, and must locate the joins where a senior citizen falls between them.

Claim -> named evidence -> analysis -> qualification:

- The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed.
- The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator.
- The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes.
- The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for.

Qualified conclusion: Fragmentation is an integration failure and not an absence of instruments, so the examination must map the statutory remedy, the five scheme components and the geriatric health route, and must locate the joins where a senior citizen falls between them.

Demand decoding: The directive **critically examine** requires a direct position on “Critically examine the claim that India's elderly-welfare architecture is fragmented across...”, every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: Fragmentation is an integration failure and not an absence of instruments, so the examination must map the statutory remedy, the five scheme components and the geriatric health route, and must locate the joins where a senior citizen falls between them.

Analytical body:

- 1. Claim and named evidence:** The Atal Vayo Abhyuday Yojana scheme page of the Department of Social Justice and Empowerment, read on 2 September 2026, records that it is a scheme to improve the quality of life of senior citizens; that its main objective is to improve that quality of life by providing basic amenities like shelter, food, medical care and entertainment opportunities and by encouraging productive and active ageing through support for capacity building of State and Union Territory Governments, non-governmental organisations, Panchayati Raj Institutions, local bodies and the community at large; and that its developmental objectives are financial security, health care and nutrition, shelter and welfare, protection of life and property of senior citizens, active and productive ageing with intergenerational bonding and skill development, accessibility, transport and an age-friendly environment, awareness generation and capacity building, promoting the silver economy of senior-friendly industrial goods and services, research and study, and project management; nothing beyond those recorded statements is claimed. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
- 2. Claim and named evidence:** The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
- 3. Claim and named evidence:** The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy,

portability or residual exclusion.

4. **Claim and named evidence:** The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: Fragmentation is an integration failure and not an absence of instruments, so the examination must map the statutory remedy, the five scheme components and the geriatric health route, and must locate the joins where a senior citizen falls between them.

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

Why this earns marks: The answer obeys the directive, explains mechanisms rather than listing schemes, uses named India-centric evidence and preserves legal, institutional, group, eligibility, implementation, fiscal, data and outcome distinctions.

How to improve this answer: For “Critically examine the claim that India’s elderly-welfare architecture is fragmented across...”, replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

ORIGINAL MAINS 4 - 15 MARKS

Question: Assess why elder abuse remains under-reported in India and what a detection-oriented policy response would require. Answer in about 250 words.

Model thesis: Under-reporting has a mechanism, so the assessment must run the typology against dependence, shame, loyalty and stigma, must treat the helpline as a detection point, and must be honest about the missing prevalence data.

Claim -> named evidence -> analysis -> qualification:

- The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational.
- The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse.

- The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists.
- The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself.

Qualified conclusion: Under-reporting has a mechanism, so the assessment must run the typology against dependence, shame, loyalty and stigma, must treat the helpline as a detection point, and must be honest about the missing prevalence data.

Demand decoding: The directive **assess** requires a direct position on “Assess why elder abuse remains under-reported in India and what a detection-oriented policy...”, every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: Under-reporting has a mechanism, so the assessment must run the typology against dependence, shame, loyalty and stigma, must treat the helpline as a detection point, and must be honest about the missing prevalence data.

Analytical body:

1. **Claim and named evidence:** The owners set out four kinds of elder abuse rather than one: physical abuse including violence and restraint; financial and property abuse including coerced transfer, theft and misuse of funds; emotional and psychological abuse including verbal aggression, isolation and humiliation; and neglect, meaning failure to provide care, extending to abandonment; and they record that financial abuse and neglect are common but under-reported; the reason the typology is examinable is that each kind has a different detection route and a different remedy, so an answer that treats elder abuse as physical violence will propose a policing response to a problem that is mostly financial and relational. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
2. **Claim and named evidence:** The owners record the mechanism of under-reporting rather than merely asserting it: abuse occurs within family settings, the victim commonly depends on the abuser for care and housing, and shame, family loyalty and fear of abandonment inhibit reporting, while the social stigma attached to institutional care makes the alternative unattractive even where informal care is inadequate; the consequence for policy is that enforcement infrastructure alone cannot surface these cases, so detection must be sensitised and proactive, and any recorded rise in reported cases may reflect improved detection as much as rising abuse. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
3. **Claim and named evidence:** The owners record Elderline as the national helpline for senior citizens, typically reached on the number 14567, providing information, grievance redress and emergency response, and the department's own site carries the same identification; the analytical point that a helpline supports is a detection point, because elder abuse occurs inside the household where the victim depends on the abuser, so a channel that does not require the senior citizen to travel to an office or to confront a family member in public is precisely the instrument that under-reporting requires, while its limitation is that it converts a call into a case only where a functioning local response exists. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction.

Qualification: State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

4. **Claim and named evidence:** The owners record the Longitudinal Ageing Study in India as the national evidence source on the health, economic and social conditions of older persons, and they attach two rules for its use: retain the wave or year with any figure quoted from it, and do not infer causation from association; alongside it they record that nationally representative and regular elder-abuse prevalence data is lacking, so policy design is data-blind on that dimension, and the owners are explicit that this is a statement about the evidence base rather than a claim about the prevalence itself. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction.

Qualification: State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: Under-reporting has a mechanism, so the assessment must run the typology against dependence, shame, loyalty and stigma, must treat the helpline as a detection point, and must be honest about the missing prevalence data.

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

Why this earns marks: The answer obeys the directive, explains mechanisms rather than listing schemes, uses named India-centric evidence and preserves legal, institutional, group, eligibility, implementation, fiscal, data and outcome distinctions.

How to improve this answer: For “Assess why elder abuse remains under-reported in India and what a detection-oriented policy...”, replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

ORIGINAL MAINS 5 - 20 MARKS

Question: Analyse the implications of India's ageing demography for income security, health care and the care economy. Answer in about 300 words.

Model thesis: Ageing changes three systems at once, so the analysis must use the recorded demographic series with its projection boundary, then test the assistance floor, the geriatric health route and unpaid family care separately before combining them.

Claim -> named evidence -> analysis -> qualification:

- The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population.
- The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard.

- The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes.
- The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for.

Qualified conclusion: Ageing changes three systems at once, so the analysis must use the recorded demographic series with its projection boundary, then test the assistance floor, the geriatric health route and unpaid family care separately before combining them.

Demand decoding: The directive **analyse** requires a direct position on “Analyse the implications of India's ageing demography for income security, health care and...”, every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: Ageing changes three systems at once, so the analysis must use the recorded demographic series with its projection boundary, then test the assistance floor, the geriatric health route and unpaid family care separately before combining them.

Analytical body:

1. **Claim and named evidence:** The umbrella-scheme page records that the number of elderly persons has increased from 1.98 crore in 1951 to 7.6 crore in 2001 and 10.38 crore in 2011, and that projections indicate the number of persons aged sixty and above in India will increase to 14.3 crore in 2021 and 17.3 crore in 2026, attributing the trend to continuing increases in life expectancy and general improvement in health-care facilities; the owners' discipline applies to that series without exception, namely that an enumerated figure and a projection are different objects, so the 1951, 2001 and 2011 figures may be cited as recorded counts while the 2021 and 2026 figures must be cited as projections recorded on that page and never as enumerated population. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
2. **Claim and named evidence:** The owners carry a precise assistance anchor: under the national old-age pension scheme, central assistance is two hundred rupees per month for eligible persons below the poverty line aged sixty to seventy-nine and five hundred rupees per month for those aged eighty and above, with State top-ups varying, and they attach two instructions: the low central floor supports an adequacy critique, and a State's higher combined amount must not be presented as the national figure; the conceptual point that must accompany the numbers is that this is an assistance floor and not an earnings-replacement pension, so it cannot be assessed against a wage-replacement standard. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
3. **Claim and named evidence:** The owners record the national programme for the health care of the elderly as the geriatric-care route inside the public-health system, and they record the age-based hospitalisation route under the national health-assurance scheme covering every citizen aged seventy and above, with the qualification that a hospitalisation entitlement addresses inpatient episodes and not the outpatient, rehabilitation, palliative and

long-term care that ageing actually generates; they also record that geriatric capacity in primary and secondary facilities is limited, so most elderly health needs are managed informally or in tertiary hospitals, which is an integration failure between the social-justice ministry and the health ministry rather than an absence of schemes.

Analysis: Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

4. **Claim and named evidence:** The owners record that formal care is provided by institutions such as old-age homes, day-care centres and professional caregivers while informal care is provided by family members and is the dominant mode in India, and that urbanisation, migration and the movement toward smaller households weaken multi-generational co-residence and therefore reduce informal-care capacity; they add that the informal-care burden falls disproportionately on women, typically a daughter or a daughter-in-law who forgoes income, and that this unpaid care is largely invisible in policy discourse, so a care-economy answer must name the carer as well as the cared-for. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: Ageing changes three systems at once, so the analysis must use the recorded demographic series with its projection boundary, then test the assistance floor, the geriatric health route and unpaid family care separately before combining them.

Executable exam-length answer / compression plan: For 20 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

Why this earns marks: The answer obeys the directive, explains mechanisms rather than listing schemes, uses named India-centric evidence and preserves legal, institutional, group, eligibility, implementation, fiscal, data and outcome distinctions.

How to improve this answer: For “Analyse the implications of India’s ageing demography for income security, health care and...”, replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

ORIGINAL MAINS 6 - 20 MARKS

Question: Evaluate the proposition that Indian policy for older persons has moved from protection towards active ageing. Answer in about 300 words.

Model thesis: Active ageing is a stated frame with concrete instruments, so the evaluation must weigh the productive-engagement and intergenerational components against the shelter and device components and close with a graded verdict rather than a uniform one.

Claim -> named evidence -> analysis -> qualification:

- The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts.

- The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator.
- The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim.
- The audited 2018-2023 Mains routing ledger routes exactly one General Studies Mains demand to this owner, the 2020 General Studies Paper II question numbered 6 on health policies for geriatric and maternal care, directive Discuss, ten marks, one hundred and fifty words, with ownership recorded as cross-cutting between this owner and the health-systems owner, and the Basic owner's answer architecture records that the elderly-care half of that demand is owned here and supersedes the older Advanced ownership; this package therefore solves the geriatric limb and states expressly that the maternal limb belongs to the health-systems owner, adds six original Mains questions with model solutions and eighty original objective questions on a strict A -> B -> C -> D rotation, and claims no other Mains or objective demand, since the audited 2024-2025 ledgers route none here and the 2018 General Studies Paper IV case study involving an elderly couple is routed by the audited ledger to the Ethics case-study owner.

Qualified conclusion: Active ageing is a stated frame with concrete instruments, so the evaluation must weigh the productive-engagement and intergenerational components against the shelter and device components and close with a graded verdict rather than a uniform one.

Demand decoding: The directive **evaluate** requires a direct position on "Evaluate the proposition that Indian policy for older persons has moved from protection...", every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: Active ageing is a stated frame with concrete instruments, so the evaluation must weigh the productive-engagement and intergenerational components against the shelter and device components and close with a graded verdict rather than a uniform one.

Analytical body:

1. **Claim and named evidence:** The owners record active ageing, associated with the World Health Organization and the Madrid International Plan of Action on Ageing of 2002, as the frame that treats continued participation in social, economic, cultural, spiritual and civic affairs as the objective rather than the mere absence of disease, and they draw the policy implication precisely: the objective is enabling productivity and engagement and not protection alone, which is why an employment-matching portal and an intergenerational-bonding component belong to the same scheme as shelter and devices, and why an answer that frames ageing purely as vulnerability has missed the frame the policy itself adopts. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction.

Qualification: State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

2. **Claim and named evidence:** The same official page records the components of the umbrella scheme as the Integrated Programme for Senior Citizens, the State Action Plan for Senior Citizens, Rashtriya Vayoshri Yojana, Senior Citizen Opportunities for Productive Engagement and the Seniorcare Ageing Growth Engine, and it describes each: the first maintains senior-citizen homes providing shelter, food, medical care and entertainment especially for indigent senior citizens; the second expects each State and Union Territory to frame its own action plan, which may comprise a five-year strategy and annual action plans, with the Union ministry releasing funds for their formulation and implementation; the third is a Central Sector scheme providing assisted-living devices to eligible senior citizens suffering from age-related disability or infirmity, the devices being of high quality and conforming to Bureau of Indian Standards specifications wherever applicable; the fourth is a portal that virtually matches the preferences of experienced senior citizens with enterprises seeking them; and the fifth identifies, evaluates, verifies, aggregates and delivers aged-care products, solutions and services to stakeholders, with the ministry acting as facilitator. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
3. **Claim and named evidence:** The owners record the assistive-device component as being implemented through the public-sector artificial-limbs corporation and as providing aids and devices to persons aged sixty and above who suffer from age-related infirmity and who are below the poverty line or have family income up to fifteen thousand rupees per month under the current guidelines, while the official page adds the quality condition that the devices must conform to Bureau of Indian Standards specifications wherever applicable; the answer-writing use of the eligibility rule is that it makes the component a targeted assistance route and not a universal entitlement of old age, so an answer must not describe it as a benefit that every senior citizen may claim. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
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Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: Active ageing is a stated frame with concrete instruments, so the evaluation must weigh the productive-engagement and intergenerational components against the shelter and device components and close with a graded verdict rather than a uniform one.

Executable exam-length answer / compression plan: For 20 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

Why this earns marks: The answer obeys the directive, explains mechanisms rather than listing schemes, uses named India-centric evidence and preserves legal, institutional, group, eligibility, implementation, fiscal, data and outcome distinctions.

How to improve this answer: For “Evaluate the proposition that Indian policy for older persons has moved from protection...”, replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.